Managing CME about Medical Marijuana: Rules, Risks, and Strategies

May 31, 2018
Posted by: Megan Peters Jovic

As medical marijuana becomes more available in communities across the country, patients and their clinicians are increasingly seeking information about its efficacy and potential use in care. Accredited continuing medical education (CME) providers can serve as a trusted source, giving clinicians the accurate information they need to deliver safe and effective care and to answer their patients' questions.

We encourage accredited providers to embrace the opportunity to address controversial topics; CME is an ideal place for discussions about evolving issues in medicine. When considering how to construct activities about medical marijuana, CME providers should apply the same prudent planning process that they do for other activities that address areas of medicine that are evolving, unproven, or experimental. It's important for recommendations to be rooted in science, evidence, and data: the validity of clinical recommendations is not determined by legislation or advocacy. It is important and appropriate to help clinicians learn *about* issues such as medical marijuana — but it's not allowable to train clinicians *how to* recommend therapies and treatments unless they are accepted by the medical profession and are based on scientifically valid evidence.

Here are key points to consider if you plan to construct CME activities about controversial topics such as the use of medical marijuana. We also provided a list of resources at the end of the article.

Relevant Rules

When managing CME about controversial topics, it may be helpful to review the Standards for Commercial Support: Standards to Ensure Independence in CME Activities and the related policies, including the CME Clinical Content Validation Policy. These rules apply whether you directly provide or jointly provide activities.

Ensure Independence from Commercial Interests

Companies involved in the production, marketing, reselling, or distributing of medical marijuana fall under the definition of an ACCME-defined commercial interest. That means that all the rules related to commercial interests apply. As described in Standard 1, all decisions about content and faculty must be made free of the control of a commercial interest. These companies are not eligible for accreditation and accredited providers cannot engage in joint providership with them. It also means that employees of medical marijuana companies, just like employees of other commercial interests, cannot participate as faculty and planners or in other roles where they are in a position to control the content of accredited CME. (For more on this rule, see this FAQ.)

Resolve Conflicts of Interest

In addition to prohibiting or constraining the role of commercial interest employees, it is your responsibility to meet the expectations of Standard 2: You need to identify and resolve conflicts of interest if anyone in a position to control content, including your planners or faculty, disclose that they have relevant financial relationships with medical marijuana companies.

Ensure Balance

Remember that when information about treatments is emerging, highly publicized, and controversial, there can be a tendency to prematurely embrace clinical recommendations that are not justified by science. It's important to ensure that your content experts review the most recent data relevant to your learners. They need to apply the same level of scrutiny to evidence about benefits as they do to harms and risks.

Be aware that many experts are also advocates, and it is your responsibility as an accredited provider to maintain the boundary between accredited CME and advocacy. Ensure that your activities present a balanced perspective and cover the range of diagnostic or therapeutic options for a given scenario. Discussion of unconventional treatments or therapies can shift the focus away from the appropriate discussion of benefits and risks of conventional management. Shifting the focus away from potentially effective care by offering an unproven alternative can be harmful to patients. Activities addressing the potential use of a controversial agent such as marijuana should present a comparison of relative merits and risks of a variety of approaches for the symptom or problem being addressed.

Take Responsibility for Joint Providership

If you engage in joint providership, you are responsible for the content of a jointly provided activity, just as you are for activities that you directly provide. You are accountable for making sure the activity adheres to requirements for independence and content validity.

Make sure that you do not jointly provide with an organization that advocates for unscientific modalities of diagnosis or therapy, or promotes treatments that are known to have risks or dangers that outweigh the benefits or are known to be ineffective in patient treatment.

No Exceptions for Content Validity

All the activities offered by accredited providers must comply with the CME Clinical Content Validation Policy. It is not acceptable to offer education that does not meet content validity standards even if those activities are not accredited or certified for credit – our clinician learners and their patients expect the content to be valid and reliable.

We're Here to Help

If you have questions about identifying or managing controversial topics or would like more information, please contact us at info@accme.org.

Resources

The Health Effects of Medical marijuana and Cannabinoids: The Current State of Evidence and Recommendations for Research (2017), published by the National Academies of Sciences, Engineering, and Medicine, presents nearly 100 conclusions related to the health effects of medical marijuana and cannabinoids use. In addition to the full report, you can download a PDF of a summary of the committee's conclusions; there are also links to other resources on report webpage.

FDA and Marijuana webpage, US Food and Drug Administration, provides an overview and links to related information including FAQs and warning letters.

Medical marijuana webpage**,** National Center for Complementary and Integrative Health, National Institutes of Health, provides an overview and links to resources for patients and clinicians.

Marijuana webpage, National Institute on Drug Abuse, National Institutes of Health, provides information on health effects of marijuana and links to resources.

"Enforcing Federal Drug Laws in States Where Medical Marijuana Is Lawful," by Lawrence O. Gostin, JD; James G. Hodge Jr., JD, LLM; and Sarah A. Wetter, JD, published in *JAMA,* April 10, 2018*,* describes the 2018 rescission of federal limits on enforcement of medical marijuana and discusses its possible consequences on state laws and clinicians. (Subscription or purchase required.)

"Cannabinoids for Medical Use: A Systematic Review and Meta-analysis," by Penny F. Whiting, PhD; Robert F. Wolff, MD; Sohan Deshpande, MSc; et al, published in *JAMA,* June 23/30, 2015, is a ***s*** ystematic review of the benefits and adverse events of cannabinoids from 28 databases.

The Role of the Physician in "Medical" Marijuana, a position paper published April 12, 2010, by the

President's Action Committee on Medical Marijuana of the American Society of Addiction Medicine, describes a series of findings, conclusions, and recommendations regarding the therapeutic value of smoked marijuana and the role of physicians in prescribing of marijuana for medical purposes.

This is the second article on managing controversial and emerging topics in CE. The first article, "Dealing with Controversial Topics in Your CE Program," is available on our website. You are welcome to share these articles with your staff, volunteers, and other stakeholders.

Source URL:
http://www.accme.org/news-publications/highlights/managing-cme-about-medical-marijuana-rules-risks-and-strategies